

2.

| CASE #      | CASE NAME                   | HEARING NAME                                                                                          |
|-------------|-----------------------------|-------------------------------------------------------------------------------------------------------|
| CVRI2203266 | HARB vs COUNTY OF RIVERSIDE | MOTION FOR SUMMARY JUDGMENT OR IN THE ALTERNATIVE SUMMARY ADJUDICATION ON 4TH AMENDED CROSS-COMPLAINT |

**Tentative Ruling:**

Parties have filed a stipulation and proposed order to be signed in Court.

3.

| CASE #      | CASE NAME                   | HEARING NAME                                                         |
|-------------|-----------------------------|----------------------------------------------------------------------|
| CVRI2203266 | HARB vs COUNTY OF RIVERSIDE | MOTION TO SET ASIDE THE COURT'S JUNE 10, 2026 JUDGMENT UNDER CCP 473 |

**Tentative Ruling:**

Parties have filed a stipulation and proposed order to be signed in Court.

4.

| CASE #      | CASE NAME              | HEARING NAME                             |
|-------------|------------------------|------------------------------------------|
| CVRI2405117 | BATISTA vs KOHUT, M.D. | MOTION FOR SUMMARY JUDGMENT ON COMPLAINT |

**Tentative Ruling:**

Motion is unopposed. The motion is granted for the following reasons:

**Factual / Procedural Context:**

This is a medical malpractice action filed against a hospital and physicians who treated Plaintiff Ericka Baptista while pregnant and in labor shortly before giving birth to her newborn Emma Obeso. A few days after birth, Emma Obeso passed away.

Plaintiffs are the surviving parents of the decedent, Emma Obeso. Plaintiffs' individual claims are being asserted in the form of a wrongful death action, seeking damages they personally suffered on the account of the decedent's death. In addition, the decedent's mother, Ericka Baptista, as the successor in interest to the decedent, is asserting, in the form of a survival action, the claims the decedent would have had had she lived, for injuries allegedly suffered prior to death. On September 11, 2024, Plaintiffs filed their Complaint, alleging the following 4 causes of action: (1) Wrongful Death

(Code Civ. Proc., § 377.60), (2) medical malpractice survival action (Code Civ. Proc., §§ 377.20, 377.30, 377.34), (3) negligent infliction of emotional distress (Ericka Baptista), and (4) negligent infliction of emotional distress (Victor Obeso). Plaintiffs allege that Ericka Baptista was 40 weeks pregnant and in labor when she was admitted to Defendants' hospital on August 26, 2023, and labored for over 24 hours while her infant became tachycardic and showed signs of fetal distress. (Complaint, ¶ 10.) Plaintiffs allege that despite these symptoms, Defendants took no steps to deliver the child but permitted Baptista to continue to labor while fetal decelerations and tachycardia continued. (Ibid.) Plaintiffs allege that Baptista and her infant were left to suffer from symptoms of severe fetal distress for several hours before being taken to the operating room for an emergent C-section. (Ibid.)

Defendant Karen G. Kohut, M.D. ("Dr. Kohut") now moves for summary judgment on the Complaint, seeking to demonstrate that Plaintiffs' action has no merit because the care and treatment Dr. Kohut provided to Plaintiff Baptista during the timeframe from approximately 8:00 p.m. on August 26, 2023, until 7:00 a.m. on August 27, 2023, was within the applicable standard of care. (Motion, 7:15-18.) Dr. Kohut argues that Plaintiffs' Complaint fails as a matter of law because they cannot establish the essential elements of breach and causation to prove negligence. The motion is unopposed.

In summary, the motion should be granted.

## **Analysis**

### **I. Legal standard.**

"A defendant moving for summary judgment has the burden of showing that a cause of action lacks merit because one or more elements of the cause of action cannot be established or there is a complete defense to that cause of action." (*Jones v. Wachovia Bank* (2014) 230 Cal.App.4th 935, 945; Code Civ. Proc., § 437c, subd. (p)(2).) "A cause of action 'cannot be established' if the undisputed facts presented by defendant prove the contrary of plaintiff's allegations as a matter of law." (*Brantly v. Pisaro* (1996) 42 Cal.App.4th 1591, 1597.) Only when defendant meets this burden, "the burden shifts to the plaintiff... to show that a triable issue of one or more material facts exists as to the cause of action or a defense thereto." (Code Civ. Proc., §437c(p)(2).)

### **II. Defendant has met her initial burden of producing evidence showing that Plaintiffs' claims lack merit.**

Dr. Kohut contends that there is no triable issue of material fact as to whether the care and treatment provided to Plaintiffs were within the standard of care and whether such care and treatment caused Emma's death. Here, Dr. Kohut presents sufficient evidence to negate the elements of breach and causation, thereby showing Plaintiffs' causes of action have no merit. (Code Civ. Proc., § 437c(p)(2) [defendant meets its burden of showing that a cause of action has no merit by showing that one or more elements of the cause of action cannot be established].)

a. *Standard of professional medical care.*

A cause of action for a health care provider's negligence consists of four elements: "(1) the duty of the professional to use such skill, prudence, and diligence as other members of his profession commonly possess and exercise; (2) a breach of that duty; (3) a proximate, causal connection between the negligent conduct and the resulting injury; and (4) the actual loss or damage resulting from the professional's negligence." (*Hanson v. Grode* (1999) 76 Cal.App.4th 601, 606.) In a medical malpractice case, expert testimony is required to prove the applicable standard of care. (*Id.* at 607.) "When a defendant moves for summary judgment and supports his motion with expert declarations that his conduct fell within the community standard of care, he is entitled to summary judgment unless the plaintiff comes forward with conflicting expert evidence...." [Citation.] (*Ibid.*) An expert witness declaration stating that defendant acted "within the standard of care," must be supported by factual explanation in forming the opinion in order to sustain summary judgment. (*Fernandez v. Alexander* (2019) 31 Cal.App.5th 770, 781; *Kelley v. Trunk* (1998) 66 Cal.App.4th 519, 524 ["an opinion unsupported by reasons or explanations does not establish the absence of a material fact issue for trial, as required for summary judgment"].)

In support of her motion, Dr. Kohut presents the supporting declaration of David Raphael, M.D. ("Raphael Decl."), a board-certified obstetrician-gynecologist, as defense expert witness who testifies as to the proper standard of medical care and causation of death. The declarant states that, in forming his opinions, he reviewed the hospital medical records, fetal monitoring strips, prenatal records, operative and delivery records, and other material customarily relied upon by physicians in his field, as well as the deposition transcripts of Ericka Baptista and Victor Obeso. (Raphael Decl., ¶ 3.) The motion was also filed with a compendium of evidence and a properly prepared separate statement of undisputed material facts. (Cal. Rules of Court, rule 3.1350(d).)

Based on his review of the medical records, the declarant provides an elaborate chronological summary of care and treatment provided to Baptista during her stay at the Desert Regional Medical Center from August 26, 2023, through August 28, 2023:

- (1) Baptista presented to the hospital with complaints of irregular contractions and was triaged by a nurse on August 26, 2023, at 7:21 p.m. (Raphael Decl., ¶ 5(1).) Baptista denied leaking or bleeding and reported positive fetal movement. (*Ibid.*) The nurse called Dr. Kohut at 7:55 p.m. and was told that the doctor was still in the operating room, so she left a message for Dr. Kohut to call Triage once she was done. (*Ibid.*)
- (2) Dr. Kohut arrived in Triage at 8:06 p.m. and was told about Baptista's complaints and history, as well as physical and urine dip results. (Raphael Decl., ¶ 5(2).) Dr. Kohut issued orders, which included "prophylactic blood pressure medicine, if indicated for pre-eclampsia." (*Ibid.*)
- (3) Dr. Kohut authored a "History and Physical" regarding her visit with Baptista at 8:00 p.m. noting as follows: Baptista had an estimated date of delivery of August 22, 2023, given her last menses; Baptista reported she was in the emergency room a few days prior for evaluation of

- ruptured membranes which came out negative; Baptista denied continued leakage of fluid and felt normal fetal movements; Baptista's contractions started in the afternoon, and she had a light headache, with some swelling in her hands and feet as well as a UTI during the pregnancy; a negative one-hour "GTT and GBS"; cervix was long, thick and 1 cm dilated with a -4 station; fetal heart tracing showing a baseline of 135 with moderate variability and accelerations; irregular contractions. Dr. Kohut further noted that Baptista should be admitted to "Labor and Delivery" and that her pre-eclampsia labs should be checked. (Raphael Decl., ¶ 5(3).)
- (4) At 10:30 p.m., Dr. Kohut evaluated Baptista and noted that "the fetal heart rate (FHR) tracing was ... a category 1 with a baseline fetal heart rate 155, with 0 decelerations and 1 acceleration." "The contraction frequency was 3 (15 min)." (Raphael Decl., ¶ 5 (4).)
- (5) On August 27, 2023, at 5:31 a.m., Baptista "spontaneously ruptured her membranes," and, at around 5:54 a.m., a nurse called Dr. Kohut to update her on Baptista's "category 1 strip, contraction pattern, and spontaneous rupture of membranes (SRM)." (Raphael Decl., ¶ 5(5).) The nurse noted that Baptista was "not feeling contractions and not cramping." (Ibid.) Dr. Kohut ordered "Cytotec 25 mcg PO." (Ibid.) Dr. Kohut did not further evaluate her.
- (6) Dr. Kohut's shift ended at 7:00 a.m. that day, and subsequently Dr. Kumar assumed the care of Baptista. (Raphael Decl., ¶ 5(6).)
- (7) At 7:34 a.m., a nurse contacted Dr. Kumar to give him "an SBRA update including the latest sterile vaginal exam." Baptista had a regular breakfast as approved by Dr. Kumar. (Raphael Decl., ¶ 5(7).)
- (8) On August 28, 2023, at 6:45 a.m., a nurse called Dr. Kumar to inform that the baby was "decelerating." Dr. Kumar "called for a C-section," and the baby was delivered at 7:01 a.m. (Raphael Decl., ¶ 5(8).) At 8:19 a.m., Dr. Kumar authored an "Operative Report," noting that "the preoperative diagnosis was chorioamnionitis and a non-reassuring fetal heart tracing from delivery" and that "[i]ntraoperatively, the fetus was found in the cephalic presentation." (Ibid.) Dr. Kumar also noted that, when the baby was delivered, "she was floppy" and "was transferred to the NICU staff and ABG's and cord blood were obtained. The Apgars were 0, 0, 0 and 3 at 1, 5, 10, and 13." (Ibid.)
- (9) On the same date, infant Emma Obeso was admitted to the NICU at Loma Linda University Hospital with "sever hypoxic ischemic encephalopathy and remained in the hospital until she passed away on August 31, 2023." (Raphael Decl., ¶ 5(9).)

Dr. Raphael opines that it is his opinion that the care and treatment provided by Dr. Kohut was within the applicable standard of care and that his conclusion is based on the following considerations:

- (1) At the time of Baptista's admission to the hospital, and "the admission cervical examination showed 1 cm dilation, a long and thick cervix, and

- a high fetal station, reflecting a very unfavorable Bishop score.” (Raphael Decl., ¶ 8.)
- (2) During Dr. Kohut’s shift, “[t]here was no clinical or laboratory evidence of preeclampsia ... and anti-hypertensive medication was never required.” (Raphael Decl., ¶ 9.)
  - (3) Baptista was “not in active labor during Dr. Kohut’s shift,” and there was “no documented cervical change attributable to contractions, and the patient was not experiencing painful or regular contractions.” It was appropriate and within the standard of care for Dr. Kohut to order “25 micrograms of Bytotec (misoprostol) for cervical ripening” because “[t]he use of Pitocin at that time would not have been appropriate given the low Bishop score.” (Raphael Decl., ¶ 10.)
  - (4) “Throughout Dr. Kohut’s shift, the fetal heart rate tracings remained Category I, the maternal condition was stable, and there were no obstetrical indications for cesarean delivery.” (Raphael Decl., ¶ 11.)
  - (5) During Dr. Kohut’s shift, “[t]here was no evidence of chorioamnionitis or intrauterine infection” as the records show Baptista was “afebrile, white blood cell count was normal on admission,” “membranes remained intact,” and Baptista was “Group B Streptococcus negative” with “no indication for prophylactic antibiotics.” (Raphael Decl., ¶ 12.)

The foregoing expert opinion shows that the care and treatment rendered by Dr. Kohut during her shift were within the standard of professional medical care. Dr. Raphael’s expert opinion as to the standard of care is supported by factual explanations. Therefore, the declaration is sufficient to meet the initial burden required to sustain a summary judgment.

The foregoing also demonstrates that the care and treatment rendered by Dr. Kohut did not substantially cause Emma’s death. According to Dr. Raphael, “Chorioamnionitis was not diagnosed until approximately 10:40 p.m. on August 27, 2023, well after Dr. Kohut’s shift ended, and “[t]he infection developed after membrane rupture and subsequent interventions, which is consistent with the known pathophysiology of ascending infection....” (Raphael Decl., ¶ 14.) Dr. Raphael further notes that Emma “ultimately suffered HIE with poor Apgar scores, and a Category 2 fetal heart tracing developed ... well after the end of Dr. Kohut’s shift.” (Raphael Decl., ¶ 16.) He further notes that “there were no warning signs, abnormal fetal tracings, or maternal findings during her care that would have required different management.” (Raphael Decl., ¶ 17.) On that basis, Dr. Kohut has met her burden of showing that subsequent diagnoses were not present during her shift despite appropriate management and treatment and therefore did not cause Plaintiffs’ injuries. (Code Civ. Proc., § 437c(p)(2) [defendant meets its burden of showing that a cause of action has no merit by showing that one or more elements of the cause of action cannot be established].)

The burden then shifts to Plaintiffs to show that a triable issue of facts exists as to the breach and causation elements of their cause of action. They fail to meet this burden because the motion is unopposed.

Furthermore, although not addressed in the motion, because Plaintiffs' negligence mal-practice claim against Dr. Kohut fails, their claims for negligent infliction of emotional distress also fail for reason a claim of negligent inflict of emotional distress is not an independent tort but a type of negligence. (*Marlene F. v. Affiliated Psychiatric Med. Clinic, Inc.* (1989) 48 Cal.3d 583, 589; *Barker v. Fox & Assocs.* (2015) 240 Cal.App.4th 333, 356-357.) Recovery for emotional distress is only available as "an item of parasitic damages" arising out of an ordinary negligence action seeking damages for physical injuries. (*Potter v. Firestone Tire & Rubber Co.* (1993) 6 Cal.4th 965, 981.) For this reason, summary judgment is appropriate as to the entire Complaint.

Therefore, the Court grants the motion for summary judgment.